

TRIAL MATCHES · DISCUSSION AID

Clinical-Trials Matching Dossier

IDH-mutant astrocytoma, grade 2 (Astrocytoma · brain.lgg) — generated 2026-07-05

Current medications: withheld in this de-identified sample

TRIALS MATCHED
48
disease filter applied · this run

BIOMARKER MATCHES
146
raw keyword/variant hits — most non-driver; see Angle 1 for graded subset

DISTINCT FINDINGS SEARCHED
10
9 molecular + tumor-type

CNS-RELEVANT ANGLE
Yes
glioma / astrocytoma keywords

- MAY FIT

 molecular / tumor-type FIT
- REVIEW

 eligibility flags to check
- CONTEXT

 disease-context only
- CAUTION

 other-tumor / pediatric / recurrent / IDH-wildtype
- RECRUITING

 open (status only)
- NOT YET RECRUITING

 pending
- ACTIVE, NOT RECRUITING

 closed to new enrolment

What this dossier is

Plain English: This is a **molecular and tumor-type FIT list**, not an eligibility decision. The pipeline compared confirmed findings from Sam's tumor (e.g. IDH1, ATRX, TP53, KDR, HLA-B) and the diagnosis (IDH-mutant astrocytoma, grade 2) against trial records. A "match" means the biology or disease keyword lines up — it does **not** confirm you qualify. Age limits, tumor grade/stability, prior treatment, and site location all still need a human review with the trial team and your oncologist. The 146 biomarker matches are **raw keyword/variant hits**: most fall on non-driver variants with no therapeutic evidence; only a small subset (below) has a credible mechanistic rationale.

Findings searched (all graded *consequence_confirmed* unless noted): ATRX_G/D, FMNL1_V/A, HEPACAM2_G/R, HLA-B_A/T, IDH1_R/C, KDR_W/*, NFKB2_R/*, TP53_-/X, WNK2_R/Q, plus *tumor_type* (disease context). Note this is a grade 2 tumor; several matches below are written for grade 3–4, recurrent, or pediatric disease and are flagged accordingly.

Angle 1 — Driver-directed (IDH1 / ATRX / TP53)

Plain English: These target the tumor's core drivers. IDH1 is the canonical driver here and Sam is already on an IDH inhibitor (vorasidenib) — some IDH-inhibitor trials use placebo or a different IDH drug, and being on one already can affect eligibility. Discuss overlap.

NCT	PHASE	INTERVENTION / TITLE	TARGET	WHY IT MAY FIT	STATUS	ELIGIBILITY FLAGS
NCT05303519	Phase 3	SIGMA — safusidenib vs placebo (IDH1-mutant glioma maintenance)	IDH1	IDH1_R/C match + tumor_type (astrocytoma condition per clinical_trials.json)	RECRUITING	REVIEW placebo arm; overlap with current vorasidenib; confirm grade-2 eligibility with site
NCT07240662	Not stated (interventional — verify)	Vorasidenib in CNS WHO grade 2 IDH-mutant diffuse glioma	IDH1	IDH1_R/C match; grade-2 specific; same agent Sam takes	RECRUITING	REVIEW single site (Heidelberg, Germany); overlap with current vorasidenib
NCT07417761	Phase 2	Tuvusertib (ATR inhibitor) in astrocytoma with ATRX mutation	ATR (synthetic-lethal w/ ATRX loss)	ATRX_G/D + IDH1_R/C both confirmed; astrocytoma condition	RECRUITING	REVIEW synthetic-lethal rationale depends on ATRX loss of function — confirm ATRX_G/D (missense) is inactivating; sites in

NCT	PHASE	INTERVENTION / TITLE	TARGET	WHY IT MAY FIT	STATUS	ELIGIBILITY FLAGS
						Spain; grade/stability criteria
NCT06794736	N/A (observational)	STRADIVARI — metabolic stratification of lower-grade glioma	IDH1 / ATRX (profiling)	Lower-grade glioma; ATRX_G/D + IDH1_R/C match	RECRUITING	CONTEXT observational / profiling; sites in Italy
NCT06575452	N/A (observational)	Epitranscriptome to diagnose/treat gliomas (tissue/blood)	IDH1 / ATRX (biology)	Glioma; ATRX_G/D + IDH1_R/C match	RECRUITING	CONTEXT sample-based; sites in France
NCT07629089	Phase 1	Vorasidenib + lomustine in recurrent IDH-mutant glioma	IDH1 (trial covers IDH1/2)	External live match; IDH1 mutation required (no IDH2 finding in Sam's data)	NOT YET RECRUITING	CAUTION recurrent disease; chemo (lomustine) added — hepatic/marrow toxicity
NCT03749187	Phase 1	BGB-290 (PARP inhibitor) + temozolomide, IDH1/2-mutant grade I–IV glioma	IDH1 + PARP	External live match; IDH1/2-mutant glioma condition (confirm grade window with site)	ACTIVE, NOT RECRUITING	CAUTION not enrolling; TMZ genotoxicity; PARP trials may require germline/HRD status
NCT03528642	Phase 1	Telaglenastat + radiation + temozolomide, IDH-mutant astrocytoma	Glutaminase (metabolic)	External live match; IDH-mutant astrocytoma condition (confirm grade window with site)	ACTIVE, NOT RECRUITING	CAUTION not enrolling; radiation + TMZ — depends on prior radiation history / cumulative CNS dose
NCT07017816	Early Phase 1	SGT-53 (transferrin-liposomal wild-type TP53 gene therapy) + checkpoint + radiation, recurrent CNS malignancies	TP53	TP53_-/X variant confirmed; TP53 gene-replacement benefit in this setting is investigational/unproven	RECRUITING	CAUTION pediatric / recurrent; likely not grade-2 adult; radiation arm — prior-radiation constraint

Angle 2 — Surface-target ADC / T-cell engager (DLL3)

Plain English: The RNA surface-target screen flagged **DLL3** as a top-percentile expression **hypothesis** (TPM 361.3; highest-ranked in the TCGA LGG/GBM cohort; markedly elevated vs the GTEx normal-brain reference — frontal cortex BA9, n=211). This is an **RNA hypothesis only**. The extreme z-value in the artifact (238.1) reflects near-zero variance in the normal-brain reference, not clinical certainty — treat it as a nomination signal, not a confirmation. Surface protein must be confirmed on tumor cells (IHC/flow) before any DLL3 therapy could be considered eligible or appropriate. All DLL3 drugs below are approved or trialed in other tumors (mainly small-cell lung cancer); CNS use is investigational.

NCT	PHASE	INTERVENTION / TITLE	TARGET	WHY IT MAY FIT	STATUS	ELIGIBILITY FLAGS
NCT06776250	Phase 2	Tarlatamab in brain cancers (astrocytic / oligodendroglial tumor)	DLL3 × CD3 TCE	DLL3 RNA very-high (nomination); conditions include astrocytic tumor	RECRUITING	REVIEW confirm DLL3 protein on tumor; CRS/ICANS boxed warning; site Ontario, Canada
NCT07243470	Phase 1/2	Tarlatamab + temozolomide, CNS tumors (adult + childhood)	DLL3 × CD3 TCE	DLL3 RNA very-high (nomination); glioma / CNS tumor conditions	RECRUITING	REVIEW confirm DLL3 protein; adds TMZ; sites in France

NCT	PHASE	INTERVENTION / TITLE	TARGET	WHY IT MAY FIT	STATUS	ELIGIBILITY FLAGS
NCT06885281	Phase 1/2	ZL-1310 (zocilurtatug pelitecan), selected solid tumors	DLL3 ADC	Broad solid-tumor language; DLL3 RNA very-high (nomination)	RECRUITING	CAUTION not glioma-specific; confirm target positivity
NCT06443489	Phase 1	SHR-4849 (topoisomerase-I ADC), advanced solid tumors	DLL3 ADC	DLL3 RNA very-high (nomination); broad solid-tumor	RECRUITING	CAUTION single China site; astrocytoma applicability investigational

DLL3 caveat: RNA expression is not proof of surface-protein targetability or trial eligibility. **DLL3 is natively expressed in normal CNS neural/progenitor tissue;** bulk-RNA elevation of a whole LGG/GBM cohort vs GTEx frontal cortex may reflect neural lineage or tissue composition rather than tumor-specific target expression. Confirm with a protein/target-specific assay on *tumor cells, not adjacent neural tissue* (IHC / flow / spatial proteomics) and a trial-site review before treating this as actionable. Tarlatamab's FDA label carries boxed warnings for cytokine release syndrome and neurologic toxicity/ICANS.

Angle 3 — Immune / vaccine / cell therapy

Plain English: These use the immune system — personalized peptide vaccines or engineered cells. IDH1 R132 produces a well-known shared neoantigen used in IDH1-vaccine trials, so the IDH1 match is a genuine rationale. **Important:** the HLA-B_A/T finding is a *somatic variant*, not a germline HLA type — peptide-vaccine eligibility depends on germline HLA typing (not assessed here), so this match should not be read as an HLA-restriction fit. Several trials are for newly-diagnosed or recurrent disease and may not fit a stable grade-2 tumor.

NCT	PHASE	INTERVENTION / TITLE	TARGET	WHY IT MAY FIT	STATUS	ELIGIBILITY FLAGS
NCT07077616	Early Phase 1	Tamavaq™ AI-designed personalized peptide vaccine, newly diagnosed glioma	Neoantigen (IDH1 shared)	IDH1/ATRX/TP53 variant matches; HLA-B_A/T noted but is a somatic variant, not HLA typing for neoantigen prediction	RECRUITING	REVIEW "newly diagnosed"; requires germline HLA typing / neoantigen workup; single site Thessaloniki, Greece
NCT06254326	Phase 1	ADAGiO — adoptive cell therapy (TTRNA-DC / TTRNA-xALT), recurrent oligodendroglioma	Tumor RNA cell therapy	IDH1_R/C match + tumor_type	RECRUITING	CAUTION oligodendroglioma + recurrent; astrocytoma may not qualify
NCT06047379	Phase 1/2	NEO212 (temozolomide–perillyl alcohol conjugate) ± ipilimumab / pembrolizumab, astrocytoma IDH-mutant	Alkylator (TMZ-POH) + checkpoint	astrocytoma IDH-mutant tumor_type match; KDR_W/* listed among findings but is <i>not</i> this drug's target (incidental biomarker match)	RECRUITING	REVIEW California sites; checkpoint-inhibitor immune toxicity (incl. hepatitis); TMZ genotoxicity

Angle 4 — Standard-of-care, imaging & other glioma context

Plain English: These matched on tumor-type or a single biomarker rather than a specific driver drug. Many are imaging, liquid-biopsy, radiation, or pediatric/RAF-pathway studies — useful context, but often not a treatment fit for a stable adult grade-2 astrocytoma.

NCT	PHASE	INTERVENTION / TITLE	TARGET	WHY IT MAY FIT	STATUS	ELIGIBILITY FLAGS
NCT04135807	Early Phase 1	Implantable microdevice in primary brain tumors (grade II–IV)	tumor_type	Grade II glioma / astrocytoma listed	RECRUITING	REVIEW requires surgery; (delayed gastric emptying) → anesthesia/aspiration precautions; Boston, MA
NCT05804227	Early Phase 1	Ulixertinib window-of-opportunity,	MAPK (tumor_type)	Glioma disease context	RECRUITING	REVIEW MAPK activation not confirmed here; Houston, TX

NCT	PHASE	INTERVENTION / TITLE	TARGET	WHY IT MAY FIT	STATUS	ELIGIBILITY FLAGS
		MAPK-activated gliomas				
NCT06381570	Early Phase 1	Vinblastine + tovorafenib, RAF-altered low-grade glioma (pediatric)	RAF (tumor_type)	Low-grade glioma context	RECRUITING	CAUTION pediatric; RAF alteration not in findings
NCT04931732	N/A (observational)	circTeloDIAG — liquid biopsy for glioma (ctDNA)	ATRX	ATRX_G/D match	RECRUITING	CONTEXT diagnostic; Bron, France
NCT07215910	Phase 3	Vorasidenib + TMZ after radiation, astrocytoma IDH-mutant grade 3	IDH1	External live match; IDH-mutant astrocytoma	RECRUITING	CAUTION grade 3 (higher than grade 2); post-radiation — prior-radiation constraint
NCT07223034	Phase 1	177Lu-PSMA-617 in gliomas	PSMA (imaging/therapy)	External live glioma match	RECRUITING	CAUTION conditions listed are mostly IDH-wildtype — poor fit for this IDH-mutant tumor
NCT03180502	Phase 2	Proton vs IMRT to preserve brain function, IDH-mutant grade II/III glioma	Radiation modality	External live match; IDH-mutant grade II/III glioma	ACTIVE, NOT RECRUITING	CAUTION not enrolling; requires radiation plan — prior-radiation / cumulative-dose constraint

Before you act on any of these

Confirm eligibility and current recruiting status directly with the trial site. Statuses shown are as-of this run (pipeline snapshot 2026-07-05) and live ClinicalTrials.gov fetch; they change often. Several trials above are written for grade 3–4, recurrent, pediatric, IDH-wildtype, or non-astrocytoma disease and are flagged **CAUTION** — a grade-2 stable tumor may not meet their criteria. Age limits were *not assessed* in the source records (min/max age null). Check germline/HLA requirements (vaccine, PARP rows), radiation history and cumulative CNS dose (radiation arms depend on radiation-naïve status and re-irradiation limits — confirm with radiation oncology), tumor grade/stability, prior-treatment washout, and whether being on vorasidenib now conflicts with IDH-inhibitor or placebo-controlled trials.

Medication interactions to raise with the care team: current meds are vorasidenib, , , , , .

- Hepatic / CYP ceiling:** vorasidenib carries a hepatotoxicity signal requiring LFT monitoring. Combined with (hepatic; CYP3A4 substrate) and (CYP2D6/CYP3A4 substrate), any added hepatotoxic agent (temozolomide, lomustine) or checkpoint-inhibitor immune hepatitis (ipilimumab, pembrolizumab) raises the hepatic ceiling. Flag current LFTs and CYP3A4/2D6-mediated interactions to pharmacy.
- Surgery / anesthesia:** (GLP-1/GIP) causes delayed gastric emptying — aspiration/anesthesia precautions and possible pre-op hold apply to surgery-requiring trials (e.g. NCT04135807 microdevice implant).
- Cell/immune therapy:** T-cell engagers (tarlatamab — CRS/ICANS risk) and checkpoint inhibitors may require washout or dose review.

This dossier does not itself compute drug–drug interaction severity; it names the axes to raise with the trial team and pharmacist.

Provenance. Run id: run_b080699235e36406. Patient context synced from triangle_pipeline initial configuration (2026-05-08).
Artifacts: **clinical_trials.json** (48 trials, 146 raw biomarker matches, disease filter applied; findings ATRX_G/D, FMNL1_V/A, HEPACAM2_G/R, HLA-B_A/T, IDH1_R/C, KDR_W/*, NFKB2_R/*, TP53_-/X, WNK2_R/Q, tumor_type — all consequence_confirmed except tumor_type=disease_context; trial-to-finding links and condition strings drawn from this file); **cell_surface_target_profile.json** (DLL3 score 88.0, TPM 361.321, log2FC 5.654, z 238.1 vs GTEx Brain Frontal Cortex BA9 n=211, tumor cohort TCGA LGG/GBM n=662, top/100th percentile); **stage6_ranked.json** (vorasidenib/ivosidenib/olutasidenib/safusidenib IDH1 gate passed; eprenetapopt TP53 — note: eprenetapopt is the ranked TP53 drug; SGT-53 above is a distinct TP53 trial asset sourced from clinical_trials.json, not from stage6).
External connectors (authoritative, cited inline): ClinicalTrials.gov live fetch — NCT03528642, NCT03180502, NCT07629089, NCT07215910, NCT07223034, NCT03749187 (statuses as returned; grade/condition detail confirmed only where the fetched record carried it). Surface-target therapy sources: FDA Imdelltra/tarlatamab label; NCI drug

dictionary; ClinicalTrials.gov NCT06776250 / NCT07243470 / NCT06885281 / NCT06443489. Diagnosis context: IDH-mutant astrocytoma, grade 2 (per run configuration); ICD-10 mapping not present in source artifacts and therefore not asserted here. Fields not present in source data are marked "not assessed" (e.g., eligibility age limits).

Not medical advice. This dossier is a discussion aid to support a conversation with **Sam's** oncologist and trial teams. A molecular or tumor-type "match" is not an eligibility determination and implies no benefit or efficacy. No treatment should be started, stopped, or changed based on this document. Every value is traceable to the run artifacts and external connectors above; where data was missing it is stated as "not assessed" rather than inferred. Verify all trial statuses and criteria directly with the sites and ClinicalTrials.gov before acting.